Expert Consultation Summary — for your review and endorsement

Research programme: EngD thesis, The Hong Kong Polytechnic University (Department of Industrial and Systems Engineering; Chief Supervisor: Prof. C. F. Cheung). Researcher: Sidney K. T. Tam. Topic: gamification and older adults' engagement with health technology, toward an adaptive-learning design framework.

Consultation: Dr Gary Lau with Ms Ada Tse (PhD student) — September 2026, approximately 30 minutes, Cantonese and English, audio-recorded with your knowledge. Date and mode: ________________________

How your contributions will be used: they enter the thesis's expert-consultation record (Appendix H) and are analysed only as professional perspectives — points of convergence and divergence with the study's findings. They are not treated as study data or as evidence for the research hypotheses.

A. Contributions recorded from Ms Ada Tse

- Older adults encountered at outreach events fall broadly into two groups: the health-engaged, who actively want risk information and practical guidance; and the resistant, who fear discovering problems they would not know how to handle.
- When an activity is framed as an assessment of their body, older adults become nervous and try to score well — the framing itself creates performance pressure.
- Games are a good vehicle for assessment: a relaxed, low-pressure setting; and games that embed everyday-life tasks may better reflect real functional ability.
- Game-based assessment must be validated against traditional therapist-administered assessments before its accuracy can be claimed.
- Deployment strengths: community centres and NGOs during regular activities; training games on the elder's own phone remove the need to travel.
- Without a pass/fail standard, the player becomes their own reference: replaying after an interval, they measure progress against their own baseline rather than an external score.

B. Contributions recorded from Dr Gary Lau

- Some patients decline even free health assessments — they prefer not to know; knowing can itself create worry.
- Information overload is a real barrier: too many apps and platforms confuse older adults. Simplicity wins (e.g., services running entirely over WhatsApp/YouTube; platform consolidation such as HA Go). “The simpler, the better.”
- Modern imaging, genetic reports and AI-generated outputs surface probabilistic and incidental findings that can frighten people who have no disease; this has grown in the AI era.
- Reporting principle for older adults: less is more — say healthy / not healthy / when to see a doctor, and route the detail to clinicians.
- Information alone is not enough and can be depressing; what works is actionable, dynamic-risk guidance with small achievable goals (e.g., “lower your blood pressure and this risk falls from 15% to 10%; walk 4,000 steps a day”).
- Guidance should come from a messenger the older person trusts — their doctor, a medical institution, or a trusted public figure.
- A Hong Kong product using mahjong and calligraphy games to screen cognitive decline is a strong example of assessment embedded in culturally familiar play — screening triage with engagement, extensible to longitudinal tracking and training. [Product name to confirm: ____________________]
- Hong Kong's current assessment pathway lacks longitudinal continuity — point-in-time results and verbal advice, with no goal-setting or tracked risk reduction (noted jointly with Ms Tse).

C. Points recorded jointly

- Group play engages older adults better than individual play, particularly for rehabilitation — consistent with Dr Sarah Yuan's earlier observation, relayed in the session and endorsed by both of you.
- Adaptive games of the kind presented (no game-over, self-referenced progress, assessment embedded in play) were viewed as potentially helpful in your fields, subject to the validation requirement above.

D. The Mirror project — please choose one option (Dr Lau)

In the session you described the Mirror concept: an avatar-based coaching device intended for group use in elderly centres — one elder performs facing it while others watch, with one camera monitoring several participants; a planned product, not yet released. Because it is unreleased, please elect how the thesis may refer to it:

☐ Option 1 — Full inclusion: the description above may appear in the thesis record, attributed to me.

☐ Option 2 — Partial: refer only to “an avatar-based group-coaching concept under development”, without product details.

☐ Option 3 — Exclude: no reference to the Mirror in the thesis.

E. Attribution — please tick your choice

Dr Lau (confirming the existing election): ☐ Continue to be identified by name in the thesis ☐ Change to anonymised

Ms Tse: ☐ I agree to be identified by name (Ms Ada Tse, PhD student) ☐ Please anonymise my contributions (e.g., “a doctoral researcher in rehabilitation technology”)

F. Endorsement

I confirm that the summary above accurately reflects my contributions in this consultation, subject to any corrections I note below. I understand how the content will be used, as described at the top of this document.

Corrections (if any): ____

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Dr Gary Lau Signature: ____________________ Date: ______________

Ms Ada Tse Signature: ____________________ Date: ______________